

Station 1A: MFR (5min)

Thoracolumbar Myofascial Release (MFR)

Possible TART Findings/Diagnosis: TART changes in the thoracolumbar region

Example Diagnosis: Increased hypertonicity and skin drag in the right thoracolumbar region

Patient Position: Prone (Supine or lateral recumbent optional)

Physician Position: Seated or standing at the side of the table



Treatment:

1. The physician places one hand over the lower thoracic spine (T10–T12) and the other over the upper lumbar spine (L1–L3). Hands may be oriented horizontally or vertically, depending on physician comfort and patient body habitus.
2. Gentle pressure is applied to engage the myofascial layer beneath each hand.
3. Fascial motion is assessed in all planes — superior/inferior, right/left, and clockwise/counterclockwise — both independently and in relation to each hand.
4. The diagnosis is made based on the combination of directions where the fascia exhibits greatest ease or restriction (e.g., inferior, right, and clockwise).
5. Treatment is applied by stacking the tissues into either the position of ease (indirect) or toward the restriction (direct), emphasizing the tension across the thoracolumbar junction.
6. The position is maintained for up to two minutes while monitoring for a release, characterized by tissue softening, warmth, creep, or unwinding. The physician may follow any tissue changes as they occur.
7. Once a release is appreciated, the tissues are gently returned to neutral and the region is reassessed for improved motion and symmetry.



Reassess

Forearm Myofascial Release (MFR)

Possible TART Findings/Diagnosis: TART changes in the forearm

Example Diagnosis: Myofascial ease in pronation, dorsal translation, and compression

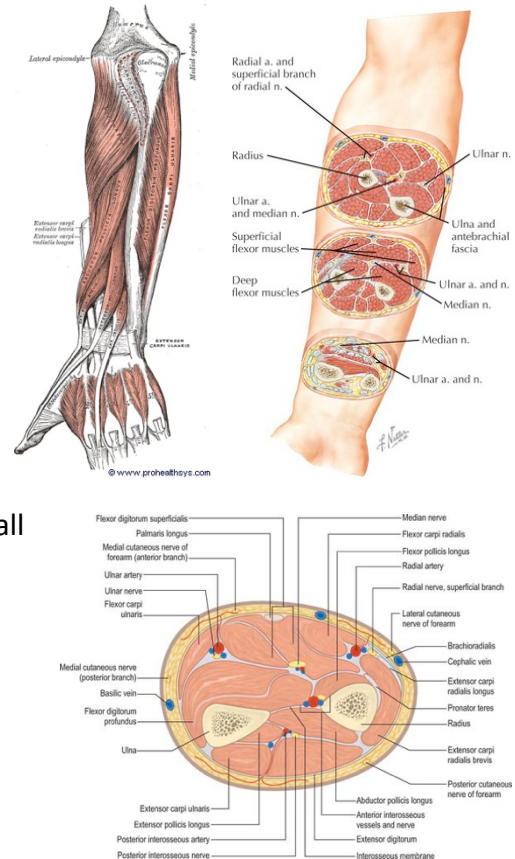
Patient Position: Seated or supine

Physician Position: Seated, facing the patient

Treatment:

1. The physician engages the forearm by placing both hands around the medial and lateral compartments, using a broad, gentle contact to access the myofascial layer.
2. Motion testing is performed to evaluate the forearm in all available planes, including pronation/supination, medial/lateral translation, dorsal/ventral translation, and compression/distraction.
3. A diagnosis is made based on the directions of ease or restriction in these motions.
4. For treatment, the physician stacks the tissues into the diagnosed position of ease (indirect) or into the restriction (direct), engaging the tissues in all planes.
5. The position is held while monitoring for a release, which may present as softening, warmth, or tissue creep. The physician may follow the tissue as it unwinds.
6. Once a release is appreciated, the forearm is gently returned to neutral and the initial diagnosis is reassessed.

Reassess



Myofascial Release (MFR) – Long Lever Lumbar and Lower Thoracic Spine with Lower Extremity

Possible TART Findings/Diagnosis: TART changes in the lower thoracic and lumbar regions, often associated with fascial restrictions through the pelvis or lower extremities

Example Diagnosis: Fascial restriction at L2–L4 with associated ease in hip internal rotation and flexion

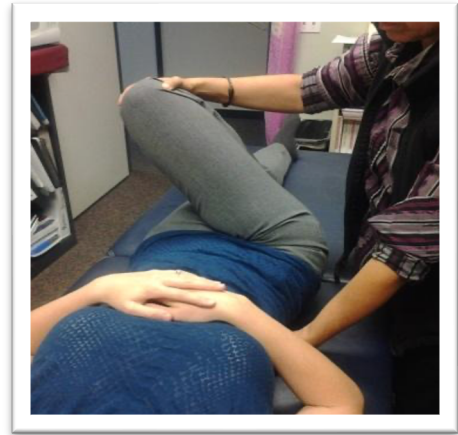
Patient Position: Supine, with the lower extremity supported in either a flexed or extended position (flexed is typically preferred)

Physician Position: Standing at the side of the table

Treatment:

1. The physician identifies the segment of greatest restriction in the lower thoracic or lumbar spine.
2. The cephalad hand is placed over the region of restriction, contacting the involved segment and surrounding myofascial tissues.

3. The caudad hand supports the patient's lower extremity, maintaining a position that is comfortable for the patient and allows for ease of motion.
4. The lower extremity is gently moved through ranges of motion — including flexion/extension, internal/external rotation, and abduction/adduction — to identify the position of maximal ease or engage the restriction, depending on the chosen approach.
5. Compression or distraction through the femur may be applied to further engage the myofascial tissues and refine the position.
6. The position is maintained until a release is palpated, which may present as tissue softening, warmth, or an unwinding motion. The physician may follow the tissue as it shifts.
7. Once a release is appreciated, the lower extremity is returned to neutral, and the region is reassessed for improved motion and tissue quality.



Reassess

Myofascial Release (MFR) – Supine Psoas

Possible TART Findings/Diagnosis: TART changes in the upper lumbar region and/or anterior hip

Example Diagnosis: Fascial ease in lumbar sidebending and external rotation of the ipsilateral thigh, suggestive of psoas hypertonicity

Patient Position: Supine

Physician Position: Seated or standing on the ipsilateral side of the psoas being treated, facing the patient's head

Treatment:

1. The cephalad hand contacts the upper lumbar spine to engage the deep myofascial tissues of the psoas. The crease of the hand is placed over the spinous processes, with the heel of the hand contacting the ipsilateral transverse processes and the fingertips contacting the contralateral transverse processes.
2. The caudad hand contacts the patient's ipsilateral thigh.
3. With the cephalad hand, the physician assesses and engages the vertebral segments in all planes: rotation, sidebending, and flexion/extension. Each plane is positioned into its respective ease.



4. Simultaneously, the caudad hand gently rotates the thigh internally and externally to identify the direction of ease at the psoas's distal attachment. The thigh is held in the position of maximal ease to facilitate shortening of the psoas.
5. Additional refinement may be achieved by incorporating hip flexion or applying gentle compression or distraction through the femur, either into or away from the psoas attachments.



6. The position is maintained while monitoring for signs of release, such as softening, lengthening, or a decrease in lumbar lordosis.

Fine-tuning through the femur allows the physician to localize force transmission toward the superior psoas attachments.

7. Once a release is appreciated and no further change is noted, the hands are relaxed and tissues are allowed to return to a neutral position.
8. The region is reassessed for changes in tissue tension and motion.

Reassess

MFR seated cervical spine with head/neck

Possible TART Findings/Diagnosis: Rt/Lt/Bilateral cervical TART changes with focus on myofascial tissue

Example Diagnosis: Rt cervical fascial pull medial, inferior, CCW

Patient Position: Seated

Physician Position: Standing behind the patient

1. The physician places one hand along the cervical region with the most significant TART changes, while their opposite hand contacts the vertex of the head
2. The physician then slowly tests superior/inferior, medial/lateral, CW/CCW to determine the directions of ease and restriction
3. The physician then stacks the planes into ease or into the restriction
4. The physician uses their cephalad hand to passively move the head and neck to achieve maximal change in the myofascial tissues
5. The physician can follow the unwind in the myofascial tissue and fine tune adjust as needed.
 - a. This technique may also be modified by adding compression, distraction and/or utilizing breath to achieve additional release in the myofascial tissues as needed
6. The physician maintains this position until a release is felt
7. The physician returns the patient to neutral
8. Reassess



MFR long lever upper T-spine with head/neck

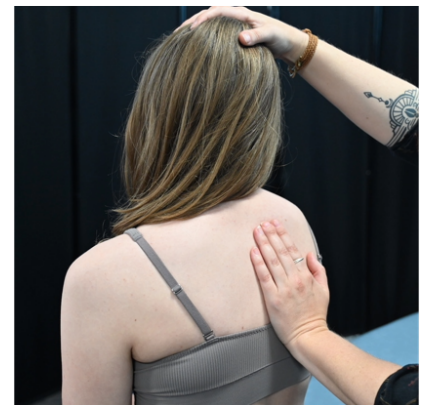
Possible TART Findings/Diagnosis: Rt/Lt/Bilateral upper thoracic TART changes with focus on myofascial tissue

Example Diagnosis: Rt thoracic fascial pull medial, inferior, CCW

Patient Position: Seated

Physician Position: Standing behind the patient

1. The physician places their hand along the Rt thoracic region with the most significant TART changes while their opposite hand contacts the vertex of the head
2. The physician then slowly tests superior/inferior, medial/lateral, CW/CCW to determine the directions of ease and restriction/barrier
3. The physician then stacks the planes into ease or into the restriction/barrier
4. The physician uses their cephalad hand to passively move the head and neck to achieve maximal change in the myofascial tissues
5. The physician can follow the unwind in the myofascial tissue and fine tune adjust as needed
 - a. This technique may also be modified by adding compression, distraction and/or utilizing breath to achieve additional release in the myofascial tissues as needed
6. The physician maintains this position until a release is felt
7. The physician returns the patient to neutral
8. Reassess



MFR long lever thoracic spine with UE

Possible TART Findings/Diagnosis: Rt/Lt/Bilateral upper thoracic TART changes with focus on myofascial tissue

Example Diagnosis: Rt thoracic fascial pull medial, inferior, CCW

Patient Position: Supine

Physician Position: Standing or seated at the side of most significant TART findings

1. The physician places one hand along the Rt thoracic region with the most significant TART changes and will monitor here while their other hand contacts the UE, cupping at the distal humerus and elbow with the patient's hand draping over the physician's forearm
2. The physician then moves the UE into F/E, aBduction/aDduction, IR/ER – stacking these vector forces through the tissues to achieve maximal ease of the arm, shoulder, torso, and ultimately the identified TART changes
3. The physician follows the unwind in the myofascial tissue and fine tune adjust as needed.
 - a. This technique may also be modified by adding compression, distraction and/or utilizing breath to achieve additional release in the myofascial tissues as needed
4. The physician maintains this position until a release is felt
5. The physician returns the patient to neutral
6. Reassess



Release of the Anterior cervical fascia

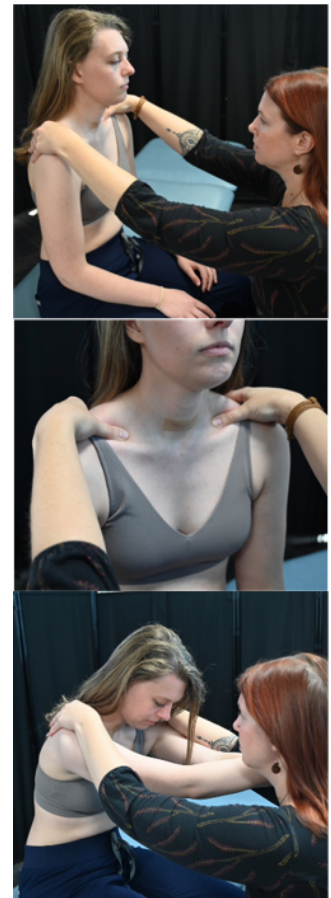
Possible TART Findings/Diagnosis: Rt/Lt/Bilateral anterior cervical TART changes with myofascial tissue, restricted motion of the thoracic inlet, lymphatic congestion

Example Diagnosis: Rt anterior cervical fascial pull lateral, rotated right

Patient Position: Seated, slightly higher than physician

Physician Position: Seated, slightly lower than patient

1. The physician places their thumbs superior to the clavicles, lateral to the SCM attachments in a posterior and inferior direction
2. The patient gently flexes forward, placing their forearms/hands on the physician's shoulders, while the physician's thumbs gently advance posteriorly and medially toward the trachea
3. The physician's thumbs are approximated just enough to engage and hold the anterior cervical fascia
4. The patient slowly straightens (one vertebra at a time), while the physician maintains engagement of the anterior cervical fascia
 - a. The patient then can slowly lift their head and neck to further stretch the fascia



- b. Optionally, the physician can instruct the patient to take a deep breath allowing the physician to further engage the fascia
5. The physician holds until a release is felt
6. The physician gently releases their contact
7. Reassess

Crossed mediastinum MFR

Possible TART Findings/Diagnosis: Restriction or ease of motion superior/inferior, Lt/Rt, diagonal or CW/CCW at different depths

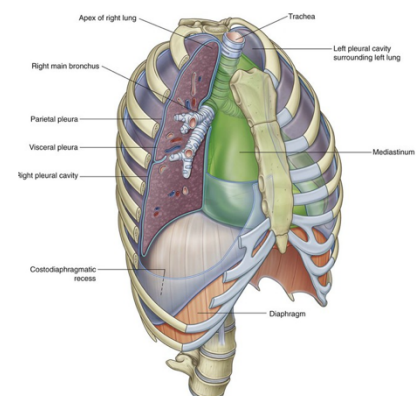
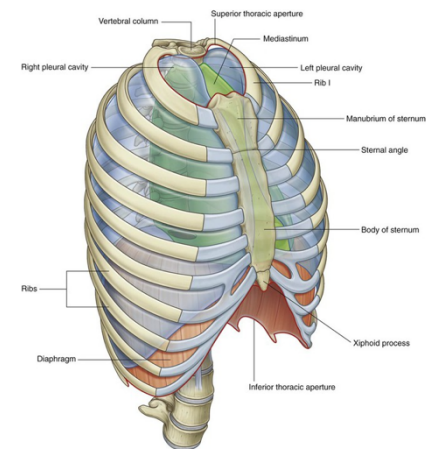
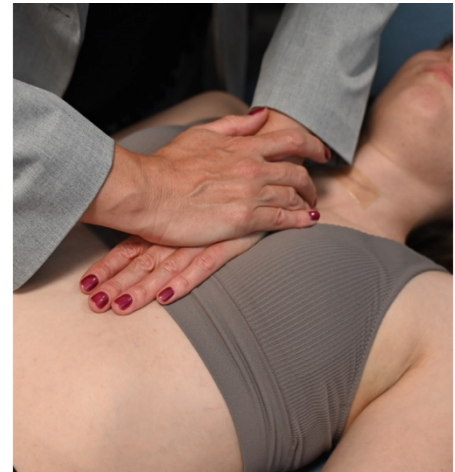
Example Diagnosis: Superficial mediastinal fascial restriction inferiorly and CCW

Patient Position: Supine

Physician Position: Standing at patient's side

Treatment:

1. The physician places their cephalad hand on the sternum on a diagonal pointing inferiorly with fingers toward the splenic flexure, and their caudad hand on top with fingers pointing toward the opposite shoulder, making an "X"
2. The physician gently and steadily compresses posteriorly, palpating the tensions as they slowly move through the layers of fascia anterior and posterior to the sternum
 - a. The physician fine-tunes their pressure, and intention to get deeper into the mediastinum, where they may detect individual structures such as the pericardium, trachea, main stem bronchi and their surrounding fascia
3. The physician then slowly tests superior/inferior, Lt/Rt, diagonal glides and CW/CCW motions
4. The physician then stacks into the ease or restriction in all planes
5. The physician holds until a release is felt
 - a. Additionally, the physician can instruct the patient to take a deep breath or add a subtle vibratory activating force to facilitate further release
6. The physician gently releases their contact
7. Reassess



Supine pelvic diaphragm MFR

Possible TART Findings/Diagnosis: Rt/Lt/Bilateral inhaled position or exhaled position pelvic diaphragm

Example Diagnosis: Lt exhaled position (exhalation) pelvic diaphragm

Patient Position: Supine w/bilateral knees flexed

Physician Position: Seated at left side of table

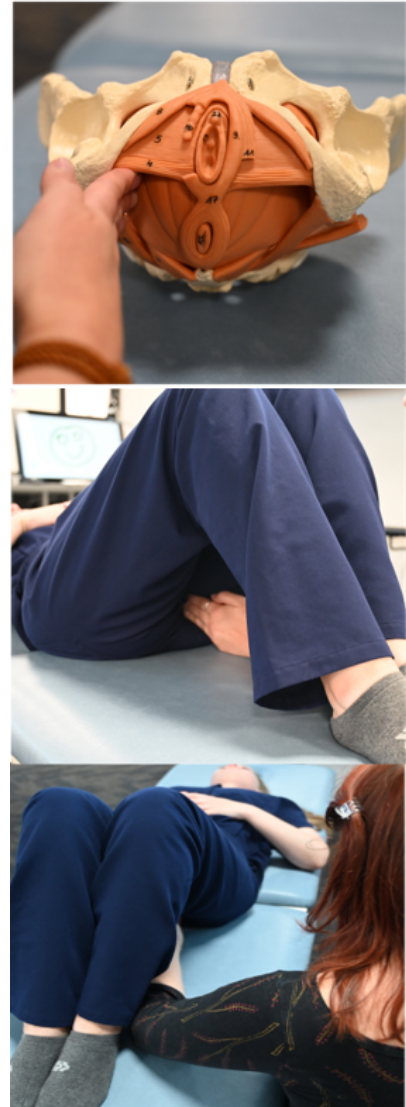
Diagnosis: Following steps 1 and 2 below. The physician then evaluates the motion of the pelvic diaphragm by asking the patient to inhale and exhale. With inhalation the pelvic diaphragm should descend and press inferiorly on the physician's fingertips. As the patient exhales the pelvic diaphragm should ascend and move superiorly away from the physician's fingertips.

If the pelvic diaphragm does not descend during inhalation this is considered an exhaled position pelvic diaphragm. If the pelvic diaphragm does not ascend during exhalation, this is considered an inhaled position pelvic diaphragm.

Treatment:

1. The physician, with a neutral wrist, contacts along the medical aspect of the Lt ischial tuberosity with their finger pads
 - a. To increase patient, comfort the physician can place a towel and/or pillow between their posterior hand and the patient's body
2. The physician then moves their finger pads just medial to the ischial tuberosity, contacting the ischiorectal fossa and gently directs a cephalad and lateral palpation of the ischiorectal fossa/pelvic floor
3. The physician engages the left restrictive barrier by moving their fingertips cephalad and lateral. The physician instructs the patient to inhale and maintains their position. This discourages further inferior movement of the pelvic floor. As the patient exhales, the physician further engages the barrier in a cephalad and lateral direction
4. The physician continues to follow steps 3 and 4 until no further release is appreciated
5. The physician slowly and respectfully removes their contact
6. Reassess both sides

Diagnosis and treatment can be combined; you can maintain contact and start treating after diagnosis



Station 1B: BLT (5min)

Short Lever Diagnosis and BLT Treatment of Tibiofemoral Joint

Possible Clinical Indications: Knee pain.

Possible TART Findings: TART changes, such as swelling, tenderness at the knee, and instability

Patient position: Seated

Physician position: Seated facing the patient

1. The physician uses their cephalad hand to monitor the tibiofemoral joint line and their caudad hand to support the patient's heel, keeping the ankle at 90 degrees, and utilizes Internal and external rotation of the tibia to compare side to side.
2. The physician then places their thumbs on the anterior tibia, just below the joint line, and their fingers in the posterior popliteal fossa and assesses the anterior/posterior and medial/lateral gliding motions of the joint.
3. Lateralize to the side of greatest restriction.
4. Chart the motions of ease, ie. Right tibial, anterior medial glide, external rotation.

Treatment of Tibiofemoral Joint SD- BLT

Patient position: Seated

Physician position: Standing at the ipsilateral side of the dysfunction knee.

Example Diagnosis: Right tibial anterior, medial glide with external rotation SD.

1. The physician's cephalad hand engages the right distal femur posteriorly, and the caudad hand engages the proximal tibia anteriorly, re-assessing anterior/posterior, Internal/external rotation, and medial lateral motions available.
2. Using either compression or distraction achieve some joint play in the articulation.
3. Next, engage each of the planes of motion to achieve a condition of balanced ligamentous tension at the articulation.
4. Once the balance point is reached, it will be held until a release is appreciated, allowing the ligaments to "unwind." Once a release has been completed, the physician returns the joint to neutral.
5. Reassess.



Short lever BLT treatment of the Cervical spine

Possible Clinical Indications: Cervicalgia

Possible TART Findings/Diagnosis: Tri-Planar vertebral finding

Patient Position: Supine

Physician Position: Seated at the head of the table, allowing their forearms to rest on the table.

Example Diagnosis: C3 RrSr

1. The physician identifies the most restricted cervical segment by individually contacting the articular pillars of each segment with their finger pads and performing a motion test for rotation by pressing the articular pillars anteriorly, one side at a time, with a vector towards the opposite eye. Sidebending may be determined by translating the vertebra with lateral to medial pressure in either direction. Test for flexion and extension by rolling their fingers superiorly on the articular pillars to test flexion and inferiorly to test extension.
2. Once the physician has determined the most restricted segment, in this case C3 RrSr, they place their second digit on the articular pillars of C4 bilaterally and their third digit on the articular pillars of C3 bilaterally.
3. Using either compression or distraction at C3 on C4 , achieve some joint play in the articulation.
4. Next, engage each of the planes of motion to achieve a condition of balanced ligamentous tension at the articulation.
5. Once the balance point is reached, it will be held until a release is appreciated, allowing the ligaments to “unwind.” Once a release has been completed, the physician returns the joint to neutral.
6. Reassess.

Short lever Diagnosis and BLT treatment of the Thoracic and Lumbar Spine

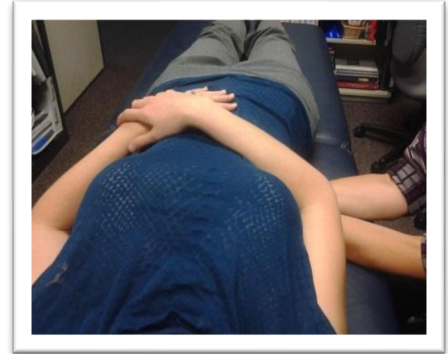
Possible Clinical Indications: Thoracic or Lumbar Pain

Possible TART Findings/Diagnosis: Hypertonicity of Thoracolumbar Muscles, Tri-Planar vertebral finding

Example Diagnosis: L3 (T3) RISI

Patient Position: Supine.

Physician Position: Seated at the side of the table.



1. The physician identifies the most restricted thoracic or lumbar segment and makes a triplanar diagnosis.
2. Once the physician has determined the most restricted segment, in this case L3 RISI, they place their cephalad hand second digit on the contralateral transverse process of L3 and their caudad hand second digit on the contralateral transverse process of L4.
3. Using their third digit, they place their cephalad hand third digit on the contralateral transverse process of L3 and their caudad hand third digit on the ipsilateral transverse process of L3. Then, place the cephalad hand fourth digit on the contralateral transverse process of L4 and the caudad hand fourth digit on the ipsilateral transverse process of L4.
4. Using either compression or distraction at L3 on L4 , achieve some joint play in the articulation.
5. Next, engage each of the planes of motion to achieve a condition of balanced ligamentous tension at the articulation.
6. Once the balance point is reached, it will be held until a release is appreciated, allowing the ligaments to “unwind.” Once a release has been completed, the physician returns the joint to neutral.
7. Reassess.

Note: The physician can adjust hand position for comfort as needed.

Long lever BLT treatment of the Cervical spine

Possible Clinical Indications: Cervicalgia

Possible TART Findings/Diagnosis: Tri-Planar vertebral finding.

Example Diagnosis: C3 RrSr

Patient Position: Supine

Physician Position: Seated at the head of the table, allowing their forearms to rest on the table.



1. Once the physician has determined the most restricted segment, in this case C3 RrSr, they place their thumb and first finger of one hand on the articular pillars of C4, and the other hand contacts the patient's parietal/frontal region.
2. Using either compression or distraction at the head at C3 on C4 to achieve some joint play in the articulation.
3. Next, engage each of the planes of motion to achieve a condition of balanced ligamentous tension at the articulation.
4. Once the balance point is reached, it will be held until a release is appreciated, allowing the ligaments to “unwind.” Once a release has been completed, the physician returns the joint to neutral.
5. Reassess.

Long lever BLT treatment of the Thoracic spine

Example Diagnosis: T3 RISI

Patient Position: Supine.

Physician Position: Seated at the contralateral side of the table.



1. Once the physician has determined the most restricted segment, in this case T3 RISI, they place their caudad hand second digit on the contralateral transverse process of T4 and their caudad hand second digit on the ipsilateral transverse process of T3.
2. Using their cephalad hand, they contact the patient's ipsilateral upper arm and, using either compression or distraction at T3 on T4, achieve some joint play in the articulation.

3. Next, engage each of the planes of motion to achieve a condition of balanced ligamentous tension at the articulation using the long lever created by the forearm contact.
4. Once the balance point is reached, it will be held until a release is appreciated, allowing the ligaments to “unwind.” Once a release has been completed, the physician returns the joint to neutral.
5. Reassess.

Note: The physician can adjust their hand position by trapping the patient's arm with their arm and moving the physician's torso to achieve the balanced tension required. Also, as the lower thoracic vertebrae, eg, T11, T12, the Lumbar long lever technique may be more appropriate.

Long lever BLT treatment of the Lumbar spine

Example Diagnosis: L3 RISI

Patient Position: Supine.

Physician Position: Seated at the contralateral side of the table.

1. Once the physician has determined the most restricted segment, in this case L3 RISI, they place their cephalad hand second digit on the ipsilateral transverse process of L3 and their third digit on the contralateral transverse process of L3 .
2. Using their caudad hand, they contact the patient's ipsilateral upper leg at or above the knee on the posterior side. Using either compression or distraction at L3, achieve some joint play in the articulation.
3. Next, engage each of the planes of motion to achieve a condition of balanced ligamentous tension at the articulation using the long lever created by the upper leg contact.
4. Once the balance point is reached, it will be held until a release is appreciated, allowing the ligaments to “unwind.” Once a release has been completed, the physician returns the joint to neutral.
5. Reassess.

Note: The physician can adjust their hand position by trapping the patient's arm with their arm and moving the physician's torso to achieve the balanced tension required.



Lumbosacral Decompression BLT

Possible Clinical Indications: LBP, pregnancy

Possible TART Findings/Diagnosis: lumbosacral compression, TART changes over lumbosacral junction

Example Diagnosis: R lumbosacral compression

Patient Position: Supine

Physician Position: Side of patient

Treatment:

1. The physician places their cephalad hand on the lower aspect of the lumbar spine (L4-L5) so that the spinous processes will fall on the palmar crease with the thenar eminence and finger tips engaged on the paraspinal musculature.
2. The caudad hand will be on the sacrum with the finger tips pointing cephalad.
3. With the cephalad hand first, move the spine into a point of balanced tension in rotation, sidebending, and flexion/extension. Then do the same with the caudad hand, moving the sacrum as if it were a vertebrae into rotation, sidebending, and flexion/extension.
(Note: the caudal contact can either go between the patient legs or from the side under the thigh depending on patient/practitioner preference and setting/context.)
4. Once both hands have positioned themselves and have found the point of balanced tension, hold until a release is palpate.
5. After releasing, reassess by inducing the same motions.



Supine SI Decompression BLT

Possible Clinical Indications: low back pain, hip pain

Possible TART Findings/Diagnosis: SI compression, decreased ROM along inferior, middle or superior axis of rotation of the SI joint

Example Diagnosis: R SI compression

Patient Position: Supine

Physician Position: Standing or seated on ipsilateral side facing patient head

Treatment:

1. Place the cephalad hand beneath the pelvis with palm up and fingertips medial to the SI joint along the joint line.
2. For short lever: place fingertips of the caudad hand just below
3. For long lever: secure the patient's ipsilateral hip and straightened knee between your caudal forearm, elbow and body wall.
4. Internally rotate the patient's hip and femur to localize forces through the leg to the ipsilateral sacrum. Using the patient's femur as a long lever, balance tensions at the SI joint, using adduction/abduction, flexion/extension and compression/distraction.
5. Once tensions have been balanced, wait for a release which may be appreciated by a softening of the tissues under the cephalad hand.
6. Slowly return the femur back to a neutral position and reassess your patient.



Long Lever



Short Lever

Supine Rib Release BLT

Possible Clinical Indications: Rib pain, upper and mid back pain

Possible TART Findings/Diagnosis: single rib restriction, inhalation/exhalation rib dysfunction

Example Diagnosis: R rib 7 posterior rib

Patient Position: Supine

Physician Position: Standing/seated ipsilateral on the side of the patient

Treatment:

1. The physician places their index finger of their cephalad hand on the contralateral aspect of the spinous process above the rib to be treated, and places their middle finger of their cephalad hand on the contralateral aspect of the spinous process on the rib below medial to the rib angle.
2. The physician then places their middle finger of their caudad hand along the dysfunctional rib with the fingertip medial to the rib angle.
3. The physician induces an anterior force with both hands to engage the segments and rib and then draws both hands laterally towards themselves. The fingers on the

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opposite side of the spinous process will rotate the vertebrae away, putting slack in the ligaments at the costovertebral junction

4. The physician continues to engage the rib along the long axis on the shaft of the rib until a release occurs.
5. Reassess.



Cephalad Hand: Index finger and middle finger on spinous process



Caudad Hand: Middle finger on shaft of rib

Sutherland Rib Technique



Possible Clinical Indications: Rib pain or thoracic back pain

Possible TART Findings/Diagnosis: single rib dysfunction or key rib in group dysfunction

Example Diagnosis: R rib 7 anterior

Patient Position: Seated at the end of the table

Physician Position: Seated next to and below the patient, ipsilateral to rib to be treated

Treatment:

1. Establish a secure contact on the dysfunctional rib and disengage the rib head by side bending the patient toward the physician in the lumbar spine, while side bending away from the physician in the thoracic spine to open the demi facets.

2. The weight of the patient should be fully supported at this point by the physician through the contact on the dysfunctional rib.
3. The rib is then translated in an anterior and posterior direction testing for ease and moving in the direction of ease to obtain balanced tension and hold this position while stacking other motions.
4. Next, ease is tested for elevation and depression keeping in mind bucket handle motion. The direction of ease is held to achieve balanced tension while stacking other planes.
5. Holding the above eases, instruct your patient to rotate their trunk away from the rib until balanced tension is achieved by the physician. The patient should keep their head erect during this motion and move it with the thorax as a unit.
6. The physician then fine tunes the previous induced motion including elevation and depression, translation anteriorly and posteriorly, and side bending away from the rib to achieve balanced tension.
7. Breath may be added as an activating force until you have achieved balanced tension.
8. Once you have reached a point of balance, you wait for a release, and once a release has been appreciated the patient can be returned to neutral and the dysfunctional rib can be reassessed.



*Remember that your contact may require navigating around the scapula and clavicle in the upper ribs.

Mediastinal BLT

Possible TART Findings/Diagnosis: Restriction or ease of motion superior/inferior, Lt/Rt, diagonal or CW/CCW over location of attachment of sternopericardial ligaments

Example Diagnosis: Superficial sternal ligamentous restriction

Patient Position: Supine

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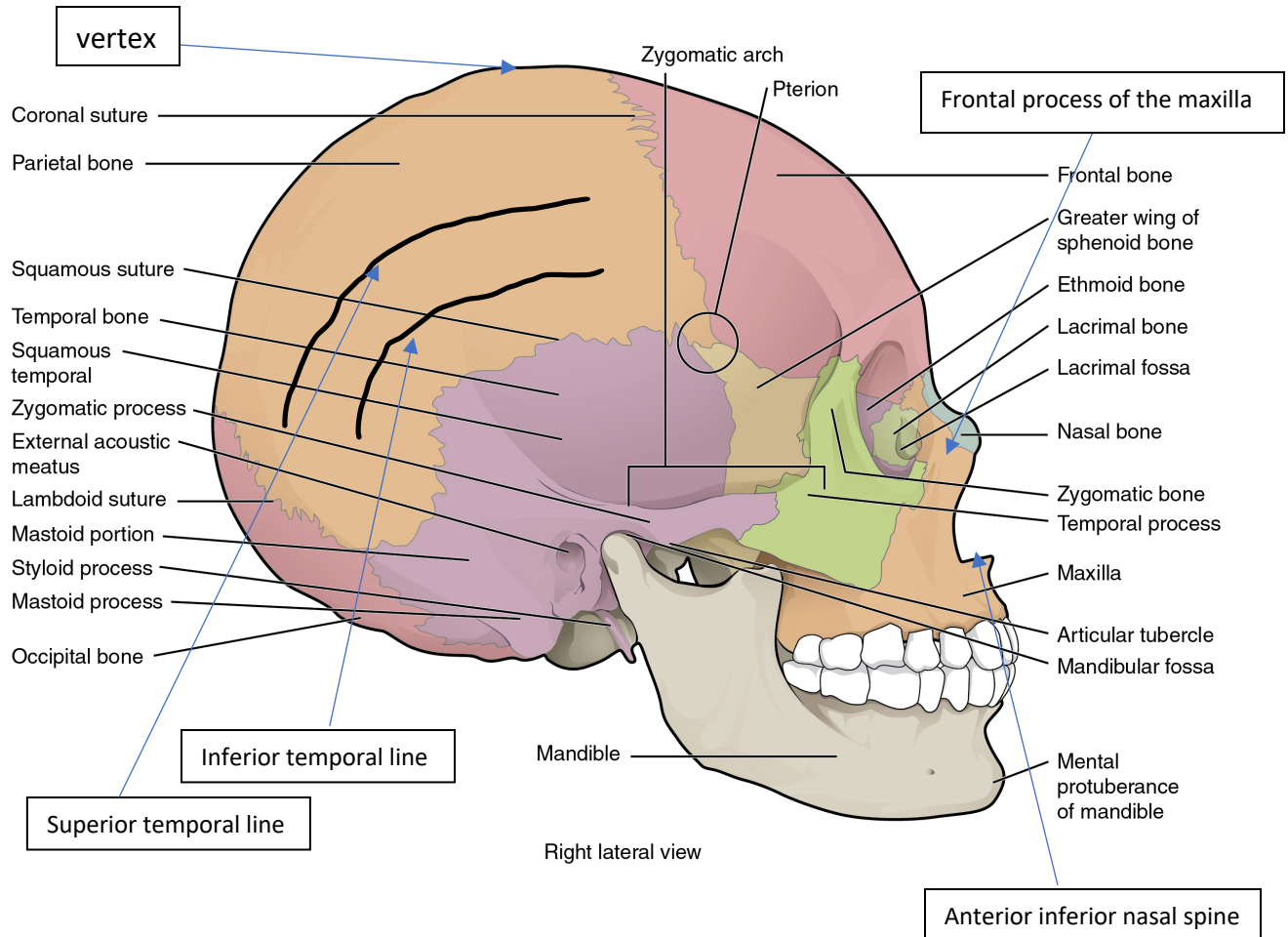
Physician Position: Standing at patient's side

Treatment:

1. The physician places their cephalad hand on the sternum over the superior sternopericardial ligament and the caudad hand on the sternum over the inferior sternopericardial ligament.
2. The physician gently and steadily compresses posteriorly, palpating the tensions as they slowly move through the layers of fascia superficial to the sternum, the sternum itself, and the ligaments deep to the sternum.
3. The physician then slowly tests superior/inferior, Lt/Rt, diagonal glides, and CW/CCW motions.
4. The physician then stacks into the ease or restriction in all planes.
5. The physician holds until a release is felt.
 - a. Additionally, the physician can instruct the patient to take a deep breath or add a subtle vibratory activating force to facilitate further release.
6. The physician gently releases their contact.
7. Reassess.

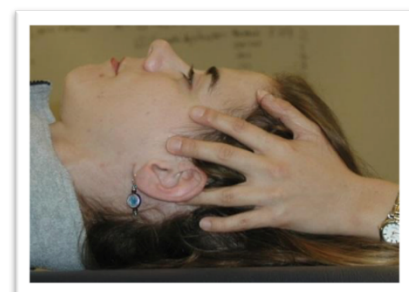
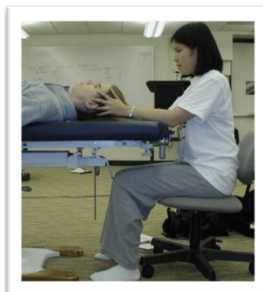


Station 2: Intro OCMM (5min)



Vault contact

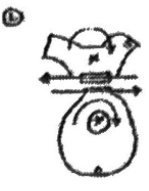
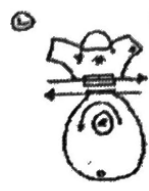
The patient should be supine with the physician seated at the head of the table facing the patient inferiorly. The physician addresses the patient with an upright posture, relaxed shoulders, feet flat on the ground, and forearms resting comfortably on the table. Bilateral palpatory contacts will include resting with minimal pressure on the frontal bones, with the first digit. Digit two, contacting the greater wings of the sphenoid. Digit three, overlying the zygomatic arches. Digit four contacting the mastoid processes of the temporal bones. And digit five, approximating the lateral angles of the occiput.



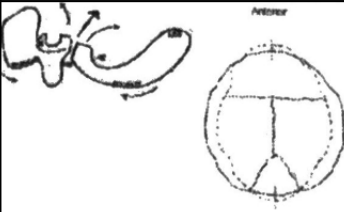
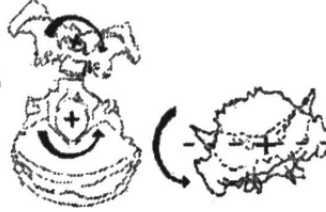
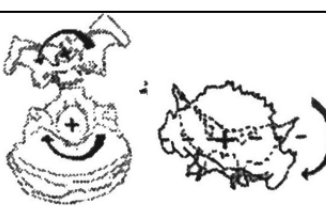
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Static Findings – The students may offer any findings they have while on the cranium in a vault hold. The lowest bar is to have two different static finding such as hard/soft, warm/cool, wide/narrow or pointing out any specific shape or texture feature using appropriate language. Some students are feeling dynamic findings, and these can be accepted with a nod and smile. Everyone will be prepped to provide two static findings at minimum.

Strain Patterns – demonstrate air hands, state axes, state rotation (opposite or same side).

Lateral strains	2 vertical axes, rotation in same direction	<u>Left Lateral Strain</u> - base of the sphenoid pointing left		5th digits point left
		<u>Right Lateral Strain</u> - base of sphenoid pointing right		5th digits point right
Vertical strains	2 transverse axes, rotation in same directions	<u>Inferior Vertical</u> base of sphenoid points inferiorly		5th digits move superiorly
		<u>Superior Vertical Strain</u> - base of sphenoid points superiorly		5th digits move inferiorly

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Strain Pattern	Axis	Mechanics	Motion of Bones	Vault Hold
Flexion	2 transverse axes (Rotation in opposite direction)	SBS moves superiorly		Narrowing of A-p diameter, widening of transverse diameter
Extension		SBS moves inferiorly		Widening of A-p diameter, narrowing of transverse diameter
Torsion	1 A-P axis (rotation in opposite direction)	<u>Left torsion</u> - left sphenoid greater wing moves superiorly		Left torsion - left index finger moves superiorly
		<u>Right Torsion</u> - right sphenoid greater wing moves superiorly		Right torsion - right index finger moves superiorly
Sidebending rotation	2 vertical axes (rotation in opposite directions) and 1 A-P axis (rotation in same direction)	<u>Left Sidebending Rotation</u> - left sided convexity		Left sided <u>convexity</u> - left side gets wider and fuller
		Right Sidebending Rotation - right sided convexity		Right sided convexity - right side gets wider and fuller